



@UNICEF Ethiopia/2024/ Demissew Bizuwork

October - November 2024

Highlights

- According to the Education Cluster, over **9 million** children are currently out of school across the country due to conflict, violence, natural disasters, and displacement; more than 6,000 schools, have closed and approximately 10,000 schools were damaged due to climate shocks and conflict.
- As of the end of November 2024, over 9 million malaria cases including 1,267 deaths were reported from Oromia (40.9%), followed by Amhara (23.5%), South Ethiopia (9.1%), and Southwest (7.5%) regions.
- In the reporting period, UNICEF has provided lifesaving treatment to more than 107,000 acutely malnourished children under five years of age across the country.
- Access to safe drinking water was provided to over 571,000 people through rehabilitation of existing non-functional water schemes and provided critical water and hygiene supplies to approximately 92,000 people.
- UNICEF requires US\$ 535 million to support the delivery of life-saving humanitarian assistance, including supplies, cash, essential services and technical assistance to refugee children, adolescents, and families. As of the end of September 2024, US\$127 million is available, leaving 76 per cent funding gap.

Funding Overview and Partnerships

UNICEF's Humanitarian Action for Children (HAC) 2024 Appeal requires US\$535.3 million to meet the critical humanitarian needs of children, adolescents, women, and men in Ethiopia. To date in 2024, UNICEF Ethiopia has received US\$95 million towards the appeal, which, when combined with US\$32.1 million of carry-over from 2023, represents only 24 per cent of the required funding needed to reach children and their families with critical lifesaving and life-sustaining support. UNICEF continues to appeal for support to ensure that children and their caregivers receive lifesaving support in 2024 and beyond.

UNICEF expresses its sincere gratitude to the many donors who have already provided critical support towards UNICEF's 2024 HAC, including the Central Emergency Response Fund (CERF), Denmark, UK Aid / Foreign Commonwealth Development Office (FCDO), US Bureau for Humanitarian Assistance (BHA), US Bureau of Population, Refugee and Migration (PRM), Japan, Canada, Ireland, Norway, German, Sweden, Denmark, and private sector donor contributions through UNICEF National Committees.

* UN Office for the Coordination of Humanitarian Affairs (UNOCHA) [2024 Ethiopia Humanitarian Response Plan](#).

** UN Office for the Coordination of Humanitarian Affairs (UNOCHA) [Ethiopia Humanitarian Needs Overview 2024](#).

*** UN Office for the Coordination of Humanitarian Affairs (UNOCHA) [2024 Ethiopia Humanitarian Response Plan](#).

****UN Refugee Agency (UNHCR) total refugees and asylum seekers in Ethiopia [as of 30 November 2024](#).

Ethiopia

Humanitarian Situation Report No. 9

including Refugee response

unicef 
for every child

Situation in Numbers



21.4 million
people in need*



12 million
children in need of
humanitarian assistance**



4.5 million
Internally Displaced People
(IDPs) ***

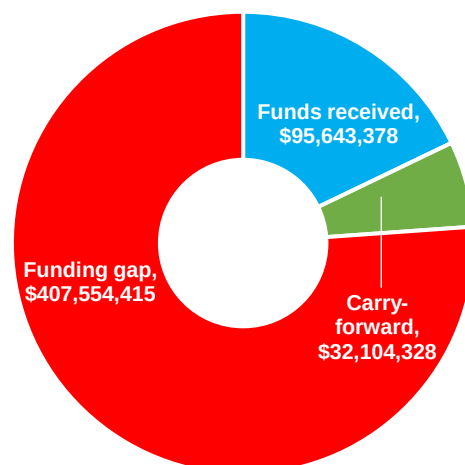


1,074,001
Total Refugees and Asylum
Seekers****

UNICEF Appeal 2024

US\$ 535 million

Funding Status (in US\$)



Situation Overview and Humanitarian Needs

The ongoing conflict in Ethiopia, particularly in Amhara and Oromia regions, is severely impacting the public health situation. Since January 2024, a total of 26,790 cases¹ and 263 deaths, resulting in a case fatality rate of 0.98 per cent. Currently, cholera outbreak is active in 18 *woredas* from six regions: Benishangul Gumuz (33.3%), Afar (27.8%), Oromia (22.2%), Somali (11.1%) and Amhara (5.6%) regions. While there has been a notable decline in cholera cases nationally, a new outbreak in Benishangul Gumuz- exacerbated by the influx of displaced individuals from Amhara - poses a significant public health threat. Although measles cases have decreased compared to last year, the risk of outbreak remains high due to the low vaccination coverage in most parts of the country which is evidenced by the high number of zero-dose children. The measles outbreak is controlled in 237 of the 249 affected *woredas* since January 2024, with active outbreaks remaining in only 12 *woredas*. Malaria has reached epidemic proportions, with over 300,000 cases² reported weekly, totalling more than 9 million cases since the beginning of 2024—the highest annual figure recorded. Besides, heavy unseasonal rainfall in early November caused flooding in Shashego *woreda* of CE region, affecting over 16,000 people and displacing more than 3,220 across 14 kebeles. Similarly, in Oromia, flooding displaced over 3,500 individuals in Gelana *woreda*, damaging 296 hectares of farmland and disrupting essential services.

The education sector has also been devastated, with approximately 9 million children out of school due to conflict, natural disasters, and displacement. Over 10,000 schools, representing 18 per cent of schools across the country have been damaged by conflict and climate shocks, further reducing the availability of safe and functional learning spaces. The highest numbers of out-of-school children are found in Amhara (4.4 million), Oromia (3.2 million) and Tigray (1.2 million) regions.

A recent SMART Plus nutrition survey in four conflict and flood-affected *woredas* of the Gambella region revealed a concerning Global Acute Malnutrition (GAM) prevalence of 17.1 per cent, exceeding the World Health Organization's 'critical' threshold of ≥ 15 per cent. Notably, Makuey and Lare *woredas* reported particularly high levels of acute malnutrition, while Jor *woreda*, reported a significantly lower rate of 4.2 per cent. Additionally, GAM prevalence varied in Kellem Wollega and South Omo zones, with rates of 7.2 per cent and 13.8 per cent, respectively. These findings highlight the urgent need for targeted interventions to combat acute malnutrition across different regions.

Summary Analysis of Programme Response³

Health

During the reporting period, 713,798 women and children accessed primary health care services including 19,137 refugees in Amhara and Gambella regions. Additionally, over 30,000 children received measles supplementation vaccinations against measles through routine and catch-up campaigns, and 30,188 live births were delivered in UNICEF-supported health facilities in Tigray, Oromia, Amhara, Gambella, Somali and Benishangul Gumuz regions. In Tigray, over 100,200 children and women accessed primary health care services, including 6,252 pregnant women who received antenatal care and 74,272 children received medical consultations. Additionally, 5,160 mothers had skilled deliveries, 6,652 mothers received early postnatal care, and immunization services continued with nearly 9,713 infants received pentavalent vaccines (Penta1 and 3), and 286 infants being fully vaccinated. In response to the cholera outbreak, UNICEF supported the oral cholera vaccination campaign and over 310,000 individuals received the cholera vaccines, representing 84 per cent of the target. In Amhara, nearly 210,000 women and children from affected communities, including IDPs and refugees/returnees, accessed primary health care services with 19,337 skilled deliveries at UNICEF-supported health facilities. Similarly, in Afar, medical consultations were provided to 51,742 drought, conflict, and flood-affected people through UNICEF-supported mobile health and nutrition teams (MHNTs).

In Oromia, a total of 213,109 individuals received lifesaving curative health service in conflict and drought-affected zones through UNICEF supported Emergency Drug Kits (EDKs) and 19,016 children aged 9-59 months received measles vaccinations in three zones. In Benishangul Gumuz, 65,247 women and children in conflict-affected *woredas* of Metekel and Kamashi zones received essential maternal, newborn, and child health (MNCH) services through UNICEF supported EDKs and Interagency Emergency Health (IEHKs). In Gambella, 26,900 individuals received essential health services, 2,780 children were vaccinated against measles, and over 2,600 livebirths were delivered at UNICEF-supported facilities. Essential health services were also provided to 2,208 people in flood-affected *woredas* of Sidama, SWE and CE regions, and 1,364 children under 15 years old were vaccinated against measles in SE region. Lastly, in Somali, 45,507 people accessed primary healthcare and 4,152 children were vaccinated against measles through UNICEF-supported MHNTs, Sustainable Outreach Strategy (SOS) and outreach teams in refugee, IDP, and host communities' sites.

¹ PUBLIC HEALTH EMERGENCY OPERATIONS CENTER (PHEOC), ETHIOPIA, November 2024

² PUBLIC HEALTH EMERGENCY OPERATIONS CENTER (PHEOC), ETHIOPIA, November 2024

³ The Summary Analysis of Programme Response covers the period from October - November 2024

As part of UNICEF's refugee response, over 19,000 individuals accessed primary health care services, 1,411 children received measles supplementation vaccination and 1,440 births were delivered in Amhara and Gambella regions. In Amhara, UNICEF continued its support for Metema refugees and returnees through MHNTs, where 2,837 individuals accessed primary health care services. Similarly, in Gambella, 16,300 women and children received primary healthcare services in UNICEF-supported facilities, 1,400 livebirths were attended by skilled health workers, and 1,411 children were fully vaccinated.

Nutrition⁴

UNICEF continued to provide lifesaving therapeutic and preventative nutrition services to IDPs, refugees, and vulnerable host populations reaching more than 107,000 children who received treatment for SAM. Additionally, over 729,000 children received vitamin A supplementation, 110,955 pregnant mothers received iron folic acid (IFA) supplementation and 450,254 primary caregivers of children aged 0-23 months received counselling on Infant and Young Child Feeding (IYCF) practices. In Tigray, the number of SAM admissions during the reporting month increased by 39.7 per cent (1,908 more cases) compared to the previous month, reflecting 50 per cent rise compared to the same period in 2023. Furthermore, a "Find and Treat" campaign was conducted in six newly accessible *woredas* in the southern zone of Tigray, screening 51,207 children, which is 85.5 percent of the targeted population. The Proxy GAM rate stands at 21.2 per cent, while the Proxy SAM was 4.1 per cent. Among 1,173 pregnant and lactating women screened, 74.9 per cent were acutely malnourished. In Amhara, SAM admissions increased by 31 per cent compared to the previous month and by 38 per cent compared to the same period in 2023, with 8.8 per cent of the total admissions requiring Stabilization Centres (SCs) due to medical complications; since January, there has been a cumulative increase of 7.6 per cent in SAM admissions.

In Afar, SAM admissions in October decreased by 21 per cent, with 959 more cases compared to the previous month, but showed a 20.5 per cent increase compared to the same period in 2023. Of the total admissions, 6.2 per cent (221 cases) were admitted to stabilization centres due to medical complications. MHNTs managed 10.8 per cent of Outpatient Therapeutic Programme (OTP) admissions in hard-to-reach remote areas, with a reporting completion rate of 54 per cent. During Community Health Days (CHD) campaign, 281,973 children under five and 95,991 pregnant and lactating women (PLWs) were screened for acute malnutrition using Mid-Upper Arm Circumference (MUAC). Among the children, 2.7 per cent (6,923) were severely malnourished, and 23.24 per cent (66,097) were moderately malnourished, while 42.32 per cent (40,622) of PLWs were acutely malnourished. The proxy GAM rate for children was 26 per cent, with 98.1 per cent screening coverage, and the malnutrition rate for PLWs was 42.3 per cent, with 96.3 per cent coverage. All identified acutely malnourished children were referred for treatment. Additionally, 98.9 per cent (292,700) of targeted children received Vitamin A supplements, 99.1 per cent (220,019) of children aged 24–59 months were dewormed.

In Oromia, SAM admissions increased by 5.1 per cent (1,260 cases) compared to the previous month and 10 per cent compared to the same period in 2023. Of the total admissions, 12 per cent (2,846 cases) required inpatient care due to medical complications. In Somali, over 8,100 children under five with SAM received treatment, reflecting a 5 per cent increase compared to the previous month but a 1.4 per cent decrease compared to the same period last year. SCs accounted for 6.3 per cent of total admissions during the reporting period. Fafan, Jarar, Shabelle and Erer zones reported 51 per cent of the regional SAM caseloads. Similarly, a total of 8,480 children under five children suffering from SAM were admitted to therapeutic feeding programmes in Sidama, CE, SWE, and SE, reflecting an overall increase of 8.6 per cent compared to the previous month. In Benishangul Gumuz, SAM admissions increased by 23 per cent (133 more cases) compared to the previous month but increased by 17.9 per cent compared to same period last year. 13.5 per cent of the admissions required referral to stabilization centres due to medical complications.

As part of UNICEF's refugee response, 1,083 children under five years and 332 pregnant and lactating women were screened, resulting in the identification of 3 cases of SAM and 35 cases of Moderate Acute Malnutrition (MAM) and linked with treatment services in Metema and Dabat refugee camps in Amhara region. Additionally, 170 children under five received Vitamin A supplementation, 121 children were dewormed, and 260 pregnant women were provided with IFA supplementation. Furthermore, 260 mothers and caregivers of children under two were counselled on optimal IYCF in emergencies. In Gambella, 22,847 children under five were screened for malnutrition, resulting in the admission of 345 children with SAM and 79 with MAM, leading to the enrolment to the OTP and Targeted Supplementary Feeding Programme (TSFP) across refugee camps. Moreover, 1,867 pregnant and lactating women were screened, with 100 found to be moderately malnourished and subsequently admitted to the TSFP. UNICEF also supported the counselling of 3,265 caregivers of children aged 0–23 months on breastfeeding, complementary feeding, and sick child feeding practices in baby-friendly spaces at nutrition centres.

⁴ Data on nutrition programme response is two months delayed due to lengthy data collection and verification process from the kebeles to federal level.

WASH

In the reporting period, UNICEF has supported over 571,000 people to get access to safe drinking water through rehabilitation of existing non-functional water schemes, water trucking, and installation of water storage tanks in Afar, Amhara, Oromia, Tigray, SE, CE, Sidama, SWE, Benishangul Gumuz, Gambella and Somali regions. Additionally, 44,800 people gained access to safe and appropriate sanitation facilities in Oromia and Tigray regions. Moreover, approximately 334,000 people were reached with handwashing and behaviour change activities in Gambella, Benishangul Gumuz, Tigray and Oromia regions. Furthermore, over 91,000 people were reached through the provision of basic WASH non-food items (NFIs) such as soaps, household water containers and water treatment chemicals, while 21 health facilities supported with construction and rehabilitation of latrine facilities and expansion of water systems, including the installation of water storage in Oromia and Tigray regions. Lastly, nearly 7,000 women and adolescent girls received support for appropriate menstrual hygiene management (MHH) during the reporting month.

In Afar, solarization of water supply schemes has been completed, benefiting 11,000 people, and WASH NFIs, including soap and Jerrycan have been distributed to cholera, flood, and drought-affected *woredas* benefitting over 14,000 people. Additionally, a total of 3,840 women and adolescent girls were reached with MHH services. In Amhara, in collaboration with partners, UNICEF provided access to safe water supply to over 191,000 people through solarization of water schemes and rehabilitation of shallow and hand-dug wells. Additionally, sanitation facilities, including accessible options for people with disabilities, were constructed at a school in North Wollo Lasta *woreda*, benefiting approximately 400 school children. Besides, a total of 5,500 people were reached with critical WASH supplies.

In Tigray, UNICEF provided water supply to nearly 30,000 people through rehabilitation and maintenance of water points, installation of pipeline connections, and the expansion and maintenance of water schemes. Furthermore, rehabilitation and desludging of sanitation facilities continued, providing 300 IDPs and host community members with access to basic sanitation. Additionally, hygiene and sanitation campaigns, including a hand-washing behaviour change programme, reached a total of 264,729. Moreover, two health facilities gained access to basic WASH services through the rehabilitation of latrines, and the installation of water storage facilities, including pipeline connections. In Oromia, over 255,000 people accessed WASH services through solarization and rehabilitation of water schemes, pipeline extensions, and the installation of Emergency Water Treatment Kits (EMWAT) and water storage facilities. Additionally, 3,467 school children in three schools gained access to WASH facilities, and over 54,000 IDPs and host community members benefitted from basic WASH NFIs, including water treatment chemicals. Furthermore, over 52,000 people were reached through hygiene promotion, cholera prevention and behavioural change interventions through various communication methods, including interpersonal outreach, radio, TV, and public gatherings. Moreover, UNICEF constructed blocks of latrines for health posts and centres, benefitting 44,500 people in three *woredas*.

In Somali, UNICEF, in partnership with Organization for Welfare and Development in Action (OWDA), rehabilitated six shallow wells and boreholes equipped with electromechanical components, including submersible pumps, solar systems, and piping, providing clean water access to 44,375 people. In Gambella, 6,375 people gained access to clean water through the maintenance of non-function water schemes in four *woredas*. Additionally, WASH NFIs, including soaps and buckets were provided to 7,810 people in flood-affected *woredas*. Moreover, in collaboration with regional health bureau (RHB), a total of 8,322 flood-affected people were reached with key hygiene and sanitation messages. Lastly, 680 women and adolescent girls received MHH services during the reporting month.

In Benishangul Gumuz, a total of 16,785 people accessed WASH services through the maintenance of water schemes in four *woredas*. Additionally, 8,350 people were reached through hygiene promotion in five *woredas* via house-to-house visits by HEWs and community volunteers in targeted *woredas* and IDP/ returnee hosting *woredas*. In SW and SWE regions, over 9,100 people gained access to potable water through solarization of three water schemes, and more than 5,800 people benefited from the distribution of WASH NFIs, including water treatment chemicals, water storages and soaps. As part of its refugee response, UNICEF, in collaboration with its partners, provided access to safe drinking water to 7,439 people in Amhara through borehole drilling and the installation of a hand pump. Additionally, UNICEF distributed WASH NFIs to benefit 10,000 refugees in the region.

Child Protection

During the month, UNICEF and its partners reached 72,640 children and 34,947 adults through various child protection and gender-based violence (GBV) interventions including case management, mental health, and psychosocial support (MHPSS), GBV prevention and response, as well as family tracing and reunification (FTR) and community-based alternative care for unaccompanied and separated children in Afar, Amhara, Benishangul Gumuz, Gambella, Oromia, Somali, SE, and Tigray regions. These efforts have been critical in improving the protection and resilience of vulnerable children and families in crisis-affected regions.

In the reporting month, a total of 19,259 individuals including 14,848 children (7,783 boys and 7,065 girls) and their caregivers 4,411 (2,876 women and 1,535 men) received MHPSS services. These include access to safe spaces positive parenting for caregivers, and basic specialized and non-specialized individual support by social workers and

psychologists deployed by UNICEF in collaboration with government and other partners across regions including in health facilities. The MHPSS helps children and caregivers to cope with the adversities of conflict, drought, and other calamities that forced them to flee from their habitual residence. Additionally, 61 individuals, including teachers, child protection officers, and social workers, received training-of-trainers (TOT) on MHPSS, Social-Emotional Learning (SEL), Psychological First Aid (PFA), trauma healing, and positive parenting in the Wollega zones of Oromia region, enhancing local capacity for sustainable support.

UNICEF-supported social workers identified and registered 1,285 (671 boys, 614 girls) unaccompanied and separated children and provided them with family tracing, reunification, and appropriate alternative care services such as kinship care arrangements. Additionally, 3,483 children that experienced different forms of violence and neglect were identified/reported and received child protection case management services through referrals to health psychosocial, and legal services. Moreover, 40,747 children and adults in Amhara and Tigray regions were reached with various awareness raising sessions on Explosive ordnance risk education (EORE) to reduce the risk of injuries and death from explosive ordnance incidents in the conflict affected sites of the regions. UNICEF and partners reached 42,813 children and adults with GBV prevention and response messages. In addition, comprehensive services including case management and referral were provided to 214 GBV survivors in Amhara, Oromia, and SE regions through UNICEF-supported One-Stop Centres (OSCs). In Afar, 18 individuals trained on national child protection case management framework, focusing on child protection and case management in the reporting month.

Through the refugee response efforts in Gambella and Somali regions, a total of 8,244 people were reached through case management, FTR and alternative care, and MHPSS services. In Gambella, 45 children (28 girls, 17 boys) experiencing psychological distress were identified and provided with psychosocial support in two refugee camps. Furthermore, community awareness sessions on gender-based violence and referral pathways were conducted in Nguenyiel refugee camp, reaching 78 individuals. To strengthen accountability to affected populations, a two-day training on managing community-based complaints and feedback mechanisms was conducted for 80 participants (17 women) in two camps. In Somali, over 3,320 children and caregivers were reached with MHPSS in Mirqan refugee camp. Furthermore, 46 unaccompanied and separated children were identified, registered, and placed in alternative care through extended family care. Moreover, 14 children who experienced violence received comprehensive case management services, including health, legal and psychosocial support. In Amhara, 400 children accessed play, learning, and recreational activities at Child-Friendly Spaces (CFS) centres.

Education

During the reporting period, UNICEF continued to support emergency education interventions, reaching nearly 48,000 new children with formal and non-formal education services in close collaboration with the Ministry of Education (MoE), Regional Education Bureaus (REBs), and NGO partners across Ethiopia. Additionally, over 103,000 children received essential learning materials to facilitate effective teaching and learning. In Afar, 1,000 out-of-school children, including 484 girls, accessed non-formal education through the Accelerated Learning Programme (ALP) and received scholastic materials. Additionally, 244 children and adolescents participated in skills development programmes, while 91 teachers and facilitators were trained in basic pedagogy which included the ALP condensed curriculum, child development, inclusive education, gender-sensitive teaching methodologies, PFA, psychosocial support (PSS), GBV and the prevention of sexual exploitation and abuse (PSEA). Moreover, 1,500 children (674 girls) who participated in the Accelerated School Readiness (ASR) programme successfully transitioned to grade one in the formal education system.

In Amhara, a total of 2,500 children were provided access to non-formal pre-primary education through ASR and ALP in conflict affected *woredas*. Additionally, 273 ASR teachers and facilitators received training on curriculum delivery, teaching materials, lesson planning, classroom management, and the use of teaching materials to enhance their skills. As part of the back-to-school campaign initiative, 11,616 school bags, 274 early childhood development (ECD) kits, 64 school in box, 22 recreational kits and sanitary pads were distributed to conflict affected *woredas*, benefiting over 15,000 children (50% girls). A total of 1,193 children (53% girls) in four *woredas* received life skills training focused on communication, confidences building, establishing, and maintaining friendships and self-discipline. Moreover, 45 teachers and education personnel were trained on emergency preparedness and response plans, Education Management Information System (EMIS), planning, monitoring, evaluation, data collection and reporting for educational development. Besides, mental health and psychosocial support training was provided to 76 teachers in conflict-affected *woredas*.

In Tigray, UNICEF and its partners facilitated access to formal and non-formal education through the ALP and ASR programmes, benefitting over 40,000 children across five *woredas*. During this period, learning materials were provided to 73,686 children including 916 children with disabilities. Additionally, 844 combined desks were distributed to 22 primary schools, benefitting 2,500 children in three *woredas*. UNICEF also supported the construction of 40 Temporary Learning Spaces in Shire, benefitting 4,440 children (2,220 girls) and repaired 20 classrooms in Adet *woreda*, benefitting 9,685 children (4,934 girls). To enhance the capacity of education personnel, UNICEF and its partners provided training

in adaptive and inclusive pedagogy to 261 teachers. Furthermore, 29,235 children (49% girls) participated in life skills sessions conducted by trained teachers and club coordinators.

In Oromia, over 2,700 children (1,362 girls) accessed formal and non-formal learning programme, while 5,273 children (2,623 girls) received individual learning materials. Additionally, 600 children and adolescent girls participated in life skills development programmes and 233 teachers and facilitators received training in basic pedagogy which will improve their instructional skills. Two MHH rooms were constructed and equipped with furniture, including tables, chairs, shelves, cupboards, and also sanitary materials, to support the attendance, retention, and learning outcomes of 646 refugee and host community adolescent girls in two refugee-inclusive secondary schools in Benishangul Gumuz. In Gambella, UNICEF supported 11,678 refugee children (4,757 girls) in primary schools by providing them with individual scholastic materials. Furthermore, UNICEF supported 1,042 children (511 girls) affected by conflict in two *woredas* of SE region, providing them with access to learning opportunities through ALP and distributing teaching materials to all children in eight schools. A total of 20 ALP facilitators (6 females) continued to implement teaching-learning processes in these schools. Additionally, school bags with basic learning materials were distributed to 2,393 children (1,115 girls) in four landslide-affected primary schools in Geze Gofa *woreda*, Gofa Zone. In Somali, 4,000 children from IDP and host communities received scholastic material, supporting four primary schools and one secondary school, and enhancing access to essential educational resources.

Social Protection

UNICEF continued to engage and support in the Ethiopia Cash Working Group (ECWG) and sub-national cash working groups, contributing to improving coordination between the humanitarian and social protection sectors, for harmonized approaches and cash transfer allocations among humanitarian actors working in cash in different regions. In the reporting month, 2,923 conflict-affected households (2,088 female-headed) in Oromia region received Shock-Responsive Cash Transfers (SRCTs). The beneficiaries were also trained on financial literacy and business skills to engage income-generating activities. To equip frontline workers and social workers with the necessary information and skills for implementing the Financial Inclusion/Humanitarian Cash Transfer (HCT) Programme, a three-day orientation and sensitization on HCT and financial inclusion was held in October for all three *woredas*. This included training on PSEA and Kobo Toolbox programme for registering selected beneficiaries. Furthermore, second round shock responsive cash transfer was provided to 1,880 flood-affected households (1,459 female-headed) in Amhara region.

In Amhara, community service workers linked 3,237 individuals to various services including health, nutrition, education, birth registration, mental health, psychosocial and legal support, income-generating activities, self-help groups. During the reporting month, the grievance committees in three *woredas* received and addressed 117 complaints, ensuring beneficiaries have a voice and promoting transparency and accountability within the social protection programme. Furthermore, 4,963 individuals had safe and accessible channels to report sexual exploitation and abuse, while 5,732 women, girls, and boys accessed GBV risk mitigation, prevention, and/or response interventions through community-level social workers during household visits, cash transfer/payment days, and regular follow-up visits. In Tigray, UNICEF conducted orientation sessions for 210 targeting committee members and 150 appeal committee members, focusing on the guidelines for humanitarian cash transfers. Additionally, UNICEF and its partner trained 39 individuals (7 female), including representatives from regional and *woreda* social affairs, committee member, and persons with disabilities associations, on disability assessment, identification, and inclusion.

Social and Behavioural change (SBC) and Accountability to Affected Populations (AAP)

During the reporting month, over 17 million people were reached and engaged through extensive outreach and mobilizations efforts during polio vaccination campaigns in many regions, as well as with messages on back to school, cholera prevention, measles, malaria, hygiene and sanitation promotion, essential health services, IYCF in emergencies, education, and GBV. In Amhara, a new engagement initiative leveraged traditional coffee ceremonies to promote key health and social practices, including full immunization for children, proper latrine usage, optimal child feeding practices, consistent school attendance and birth registration.

With UNICEF's financial and technical support, 1,247 frontline workers, community health workers, village health leaders (VHLs) and health extension workers received training on utilizing PICO projectors for engagement and SBC skills, focusing on interpersonal communication, risk communication and community engagement. Furthermore, apart from disseminating information, ensuring participation in preventive behaviours and available services, efforts were made to gather feedback from over 13,000 people regarding the services provided. The main concerns included a shortage of drugs and the closure of health facilities during working hours in Benishangul Gumuz and a lack of access to shock-responsive cash support, food, shelter/protection, and clean water in Amhara region. Feedback was collected through established AAP mechanisms, such as focus group discussions, community sessions, peer-to-peer dialogue, and house-to-house and tent-to-tent visits.

Protection from Sexual Exploitation and Abuse (PSEA)

UNICEF contributed to the National PSEA network by presenting findings from its PSEA risk assessment in IDP operations, particularly in Tigray and by representing the network at a national emergency response planning workshop where PSEA activities, targets, and indicators were identified and presented for inclusion in the 2025 National Humanitarian Response Plan. In the reporting period, UNICEF reached over 39,000 individuals with information on safe and accessible SEA reporting channels, through focus group discussions, community awareness-raising sessions, and information campaigns. The messaging included dissemination of safe reporting channels for SEA allegations, UNICEF's zero tolerance for SEA, community entitlements and rights on access free humanitarian assistance. Partner PSEA capacity assessments have been conducted to evaluate UNICEF's implementing partners ability to prevent and respond to SEA, identify needs, and develop action plans to address gaps and enhance their PSEA capabilities. Notably, 61 per cent of the partners were assessed as low risk. Furthermore, 147 UNICEF staff and 94 partners in Amhara, Oromia, Somali and Tigray received refresher training on PSEA concepts, prevention and response strategies, UNICEF commitments, programming for PSEA and monitoring and reporting of SEA incidents.

In Amhara, UNICEF in collaboration with its partner, trained 66 humanitarian workers on PSEA and AAP to enhance their capacity to prevent exploitation, improve accountability, and integrate PSEA and AAP principles into humanitarian operations, ensuring safer, more effective responses in the region. Additionally, 5,105 individuals (55% children under 18) were reached through various UNICEF activities with information on how to report incidences of SEA safely and confidentially by humanitarian aid workers through existing reporting channels. Similarly, in Benishangul Gumuz, key messages and platforms were provided to 178 children and 858 adults in Bambasi IDP site to voice their concerns about preventing sexual exploitation and abuse, while also informing them about available services and reporting mechanisms. In Afar, information on reporting SEA and GBV cases was prominently displayed in all Accelerated Learning Programme centres and target schools, enabling safe and confidential reporting. Community case workers conducted ongoing awareness sessions, highlighting the significance of the PSEA and the reporting system.

In SE, UNICEF in collaboration with Women Empowerment-Action (WE-Action), trained 23 community volunteers and project staff from three *woredas* on PSEA, PSS, and GBV reporting mechanisms. Additionally, as part of the HCT programme implementation across four *woredas*, an orientation session was held for government frontline workers, which included a PSEA session attended by 50 adult participants.

Human Interest Stories and External Media

In the reporting period, UNICEF hosted [Lord Collins of Highbury](#), UK Minister for Africa, and visited the UNICEF and WFP-supported lifesaving services, provided to mothers, young girls, and children in the Tigray region of Ethiopia. UNICEF published a video to promote [proper nutrition](#) for child development, emphasizing [Ready to Use Therapeutic Food](#), alongside videos highlighting [social worker](#) roles in protecting children from harmful practices. Furthermore, UNICEF published videos on the role of [solarization for health care centres](#) and provide access to [clean water](#) for remote communities. Additionally, videos on [menstrual hygiene](#) as well as content on [accelerated education programme](#) for out of school children and [support for schools](#) in drought affected areas were also produced. Content was also made on [psychosocial support for refugee children](#).

UNICEF published human interest stories highlighting the [accelerated learning programmes](#) in conflict affected regions and how [education](#) can be a lifeline for children in emergency settings; the impact of SBC programmes on [handwashing](#) and [hygiene practices](#); the role of [health workers in providing vaccines](#) in pastoralist communities; the impact of [cash transfer assistance](#) to rural communities; the story of a [family adopting on a child refugee](#) from South Sudan ;community members role and impact in ending [Female Genital Mutilation](#) and [child marriage](#); the positive impact of access to [maternity care services](#) in different regions of Ethiopia.

In the months of October and November, UNICEF led two communication campaigns, one for [COP29](#) where UNICEF sponsored two youth advocates to participate in the COP29 forum in Baku. The youth advocated documented their work and the other for [16 Day of Activism Against GBV](#). Additionally, UNICEF was also a key sponsor of the annual [Great Ethiopian Run](#) and the Children's race and advocated for proper nutrition. Finally, UNICEF also created awareness on the different international advocacy day in the months of October and November including: [World Mental Health Day](#), [International Day of the Girl](#), [Global Handwashing Day](#), [World Food Day](#), [World Polio Day](#), [World Pneumonia Day](#), [World Children's Day](#), and [World Teacher Day](#).

Donor contributions to our humanitarian appeal for children were also highlighted on social media including [Gavi](#), [the European Union](#), [Gates Foundation](#), [Ministry for Foreign Affairs of Finland](#), [KFW](#), [Eleanor Crook Foundation](#), [the End Fund](#), [Rotary International](#), [Power of Nutrition](#), [KOICA](#), [Auswärtiges Amt](#), [UN CERF](#), [EU ECHO](#), [MOFA of Japan](#), [FCDO](#), [Ministry of Foreign Affairs Denmark](#), [US Department of State PRM](#), [Global Affairs Canada](#).

For more content please check: [Facebook](#), [Twitter](#), [YouTube](#), [LinkedIn](#), [Instagram](#), and [www.unicef.org/ethiopia](#).

Humanitarian Leadership, Coordination and Strategy

UNICEF's humanitarian strategy is aligned with the 2024 HRP and UNICEF cluster and programme priorities. The humanitarian response in Ethiopia is led by the Ethiopian Disaster Risk Management Commission (EDRMC) through the federal and regional Disaster Risk Management Technical Working Groups (DRMTWGs), in which UNICEF remains an active contributing member. At the national and subnational levels, UNICEF continues to lead the Nutrition and WASH Clusters and Child Protection Area of Responsibility (AoR) and co-leads the Education Cluster supporting relevant line ministries and the national and regional level DRMCs. As the lead agency for three clusters and one AoR, UNICEF brings leadership to intersectoral coordination and efficiency and effectiveness when responding to immediate needs. UNICEF continued active participation in OCHA-led regional and sub-regional coordination mechanisms that involve the UN, NGOs, and the Government of Ethiopia and that take place through the Emergency Coordination Centre (ECC) meetings, using these platforms to advocate for the needs of affected populations across all regions. Overall, UNICEF's leadership, coordination, and strategy in Ethiopia aim to ensure a well-coordinated and efficient response to humanitarian needs, leveraging partnerships and collaborating with relevant stakeholders for maximum impact.

For more content please check: [Facebook](#), [Twitter](#), [YouTube](#), [LinkedIn](#), [Instagram](#), and www.unicef.org/ethiopia

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Annex A - Summary of Programme Results⁵

	Cluster/Sector Response		UNICEF and IPs Response (Overall response including Refugee Response)		UNICEF and IPs Response (Refugee Response only)	
Sector	2024 target	Total results	2024 Cumulative target	Total Cumulative results	2024 target	Total results
Nutrition ⁶		Jan – November 2024		Jan – November 2024		Jan – November 2024
Number of children aged 6 to 59 months with severe wasting admitted for treatment	965,888	605,117	965,888	605,117	3,990	4,877
Number of children aged 6 to 59 months receiving Vitamin A supplementation (SEMESTER 1&2)			5,815,199	5,530,323	43,955	27,290
Number of primary caregivers of children aged 0 to 23 months receiving IYCF counselling ⁷	1,053,135	321,500	1,053,135	321,500	24,786	3,642
Number of pregnant women receiving preventative iron supplementation			1,339,067	891,775	30,813	5,945
Health						
Number of children and women accessing primary healthcare in UNICEF supported facilities			2,876,553	3,537,054	70,500	96,482
Number of children below 15 years of age vaccinated against measles			480,000	1,730,549 ⁸	40,000	3,856
WASH						
Number of people accessing a sufficient quantity and quality of water for drinking and domestic needs	6,351,065	4,059,090	4,896,984	2,814,428	71,868	31,532
Number of people accessing appropriate sanitation services	901,638	740,752	1,001,160	241,968	44,240	1,272
Number of people reached with hand-washing behaviour-change programmes	6,047,211	3,043,881	7,247,012	1,066,429	145,000	15,580
Number of people reached with critical WASH supplies	3,168,874	1,599,273	3,155,124	1,235,691	79,200	29,100
Child Protection						
Number of children, adolescents and caregivers accessing community based MHPSS	350,000	170,324	188,014	170,403	6,513	28,624
Number of UASC provided with alternative care and/or reunified	15,000	9,857	13,205	8,037	1,099	413
Number of girls and boys who have experienced violence reached by health, social work, or justice/law enforcement services	70,000	23,795	49,506	23,239	3,975	328
Education						

⁶ Data on nutrition programme response is two months delayed due to lengthy data collection and verification process from the kebeles to federal level.

⁷ To avoid cumulative reporting for this indicator, monthly results are reported to prevent double counting and we take the maximum number by the end of the year.

⁸The over-achievements in measles vaccination are due to reactive campaigns conducted in selected *woredas* in January 2024.

	Cluster/Sector Response		UNICEF and IPs Response (Overall response including Refugee Response)		UNICEF and IPs Response (Refugee Response only)	
Sector	2024 target	Total results	2024 Cumulative target	Total Cumulative results	2024 target	Total results
Nutrition ⁶		Jan – November 2024		Jan – November 2024		Jan – November 2024
Number of children accessing formal and non-formal education, including early learning	746,119	138,900	717,398	302,644 ⁹	30,000	15,460
Number of children receiving learning materials	700,000	934,160	517,575	362,114	19,200	34,849
Social Protection						
Number of households reached with UNICEF-funded humanitarian cash transfers			265,000	10,949	15,000	84
PSEA						
Number of people with safe and accessible channels to report SEA by personnel who provide assistance to affected populations (Cross-sectoral)			1,224,246	264,974	17,967	23,670
GBVIE						
Number of women, girls and boys accessing GBV risk mitigation, prevention and/or response interventions (Cross-sectoral)			210,801	262,970	8,190	20,883
Social Behaviour Change (SBC)						
Number of affected people (children, caregivers, community members) reached with timely and life-saving information on how and where to access available services			42,713,214	39,457,741	145,000	27,505
Number of people engaged in reflective dialogue through community platforms			4,588,883	7,017,367	72,327	2,220
Number of people sharing their concerns and asking questions through established feedback mechanisms			289,625	112,596	7,232	-

⁹ The over achievement for this indicator is because of the UNICEF result include formal and non-formal learning and refugee response while education cluster collects only the children reached through non-formal learning. Additionally, there was some double counting by partners since humanitarian response are continued for multiple months. Data has been revised during the mid-year reporting.

Annex B - 2024 HAC Funding Status including Refugee Response

Sector	2024 HAC Funding Requirements (US)	Funds available			Funding gap	
		Humanitarian resources received in 2024 (USD)	Resources available from 2023 (Carry-over) (USD)	Total Funds Available (USD)	\$	%
Health	45,002,047	15,141,988	5,338,155	20,480,143	24,521,904	54%
Nutrition	99,826,914	30,884,776	9,467,966	40,352,742	59,474,172	60%
Child Protection, GBViE, PSEA	59,410,938	4,363,759	3,661,169	8,024,928	51,386,010	86%
Education	60,000,072	3,041,177	3,467,824	6,509,001	53,491,071	89%
WASH	144,280,947	21,534,589	7,420,197	28,954,786	115,326,161	80%
Social Policy	105,551,000	9,448,141	2,749,017	12,197,158	93,353,842	88%
Cross Cutting (SBC, AAP, RCCE)	21,230,203	11,228,948	0	11,228,948	10,001,255	47%
Total	\$535,302,121	\$95,643,378	\$32,104,328	\$127,747,706	\$407,554,415	76%